This Patient Group Direction (PGD)is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction for the administration of Flucloxacillin for the treatment of Cellulitis. If this is unsuitable, or the person has a penicillin allergy, for the administration of Clarithromycin or Doxycycline for the treatment of Cellulitis

Patient Group Direction, version 007, issued 30 September 2026. Supersedes Version 006, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE / NICE CKS Guidelines for Cellulitis

Overview

- **Cellulitis** is a **bacterial infection of the skin and subcutaneous tissues**, usually caused by **Streptococcus pyogenes** or **Staphylococcus aureus**.
- Commonly affects the **lower limbs** but can occur anywhere.
- Presents with **redness, swelling, warmth, and pain**, often with **systemic features** (e.g. fever, malaise).

Assessment

Clinical Features

- Erythema, oedema, local warmth, tenderness.
- May have systemic symptoms: fever, chills, lymphangitis.
- Unilateral involvement is typical; bilateral leg redness suggests **alternative diagnoses** (e.g. venous eczema, oedema).

Consider Differential Diagnoses

- DVT, contact dermatitis, venous eczema, gout, insect bites, lipodermatosclerosis.

Severity Assessment

- **Mild**: Localised symptoms, no systemic signs.
- **Moderate**: Spreading infection with systemic symptoms.
- **Severe**: Rapid progression, systemic toxicity, immunosuppressed, or failing oral antibiotics.

Red Flags (Urgent Referral or Admission)

- Severe systemic illness or sepsis.
- Suspected **necrotising fasciitis** (pain out of proportion, rapid spread, skin necrosis).
- Periorbital cellulitis or orbital involvement.
- Cellulitis in immunocompromised or diabetic foot.

Management

Antibiotic Treatment (Empirical)

Follows **NICE antimicrobial prescribing guidance**:

Severity	First-Line Antibiotic	Notes
Mild (oral)	**Flucloxacillin 500 mg QDS for 5–7 days**	First-line for non-severe cases
Penicillin allergy	**Clarithromycin 500 mg BD** OR **Doxycycline 200 mg stat, then 100 mg OD**	Avoid in pregnancy
Moderate–severe or systemic signs	**IV flucloxacillin** (or hospital protocol)	Consider hospital admission

- Duration: 5 days, review at 48 hours, extend if needed.
- Mark margins of erythema to assess spread.
- Elevate affected limb to reduce swelling.

Adjunct Management

- Analgesia (e.g. paracetamol or NSAIDs).
- Rest and elevation.
- Compression therapy once acute infection resolves (to reduce recurrence in lower limb cellulitis).

Recurrent Cellulitis

- Address predisposing factors: toe web fungal infection, venous insufficiency, obesity, skin trauma.
- Consider prophylactic antibiotics (e.g. low-dose penicillin V) if ≥**2 episodes/year** despite preventive measures.

Follow-Up and Review

- Reassess within 48 hours to check response to antibiotics. Under this PGD the reassessment is booked at the supplying pharmacy before the patient leaves and is performed in person by a pharmacist.
- Refer or escalate if:
    - No improvement
    - Deterioration
    - Unclear diagnosis

Patient Group Direction

for the supply/administration of

Flucloxacillin 500mg or 250mg capsules

for the treatment of

Cellulitis

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Flucloxacillin is indicated for the treatment of MILD cellulitis (Eron class I: no signs of systemic toxicity and no uncontrolled comorbidity) of a limb or the trunk in adults aged 18 years and over, caused by susceptible Gram-positive organisms such as Staphylococcus aureus or Streptococcus species. Moderate or severe cellulitis (Eron class II to IV) is not covered by this PGD.
Inclusion criteria	- Adults aged 18 years or older. - MILD cellulitis (Eron class I) of a limb or the trunk: localised erythema, warmth, swelling and pain, with NO fever, NO tachycardia, NO hypotension, NO confusion and NO rapidly spreading margin. - The margin of the erythema has been marked and the time recorded, so that spread can be judged at review. - A reassessment at 48 hours at the supplying pharmacy has been booked before the patient leaves, and the date and time recorded. - No known allergy to penicillins. - Informed consent obtained.
Exclusion criteria	- Known allergy or hypersensitivity to penicillins or beta-lactam antibiotics. - Any sign of systemic illness or sepsis: temperature 38°C or above or below 36°C, heart rate above 90, respiratory rate 20 or above, systolic blood pressure below 100, new confusion, or rigors (call 999 or send to A&E). - Suspected necrotising fasciitis: pain out of proportion to the appearance, rapid spread, skin discolouration, crepitus or blistering (999). - Periorbital, orbital or facial cellulitis (same-day urgent referral). - Cellulitis of a diabetic foot, or in a limb with lymphoedema or chronic venous ulceration (same-day GP referral). - Immunosuppression, including chemotherapy, biologics, long-term oral steroids, or poorly controlled diabetes. - Cellulitis following an animal or human bite, or with fresh water or sea water exposure (different organisms; refer). - Suspected deep vein thrombosis, or redness of both legs (usually not cellulitis; refer). - Abscess requiring drainage or infected wound needing surgical review. - Not improving after 48 hours of an antibiotic, or a second episode at the same site within 3 months (refer). - Hepatic dysfunction or history of flucloxacillin-associated jaundice. - Taking any medication with a clinically significant interaction. - Severe renal failure with creatinine clearance less than 10ml/min
Cautions	- Use with caution in patients with renal impairment or history of hepatic issues. - Advise patients to complete the full course and report any side effects.

	- Consider Clostridioides difficile risk in patients with recent antibiotic use or hospitalisation. - Pregnancy and breastfeeding: flucloxacillin may be supplied where clinically indicated. Flucloxacillin is the NICE NG141 first-line choice in pregnancy and the SmPC does not contraindicate it in pregnancy or breastfeeding; this matches the Skin and Soft Tissue Infection PGD. Record that the patient is pregnant or breastfeeding and the indication. The clarithromycin and doxycycline arms remain excluded in pregnancy and breastfeeding. - The 48-hour reassessment is performed in person by a pharmacist at the supplying pharmacy and is booked as an appointment before the patient leaves; a phone call is not sufficient. At the reassessment record whether the erythema is within or beyond the mark, the temperature, whether pain has improved, and the decision. Spread beyond the mark, no improvement, deterioration or an unclear diagnosis is a same-day referral. If the patient does not attend, contact them the same day; if they cannot be reached, record the attempt and inform the GP. - Caution is advised when flucloxacillin is administered concomitantly with paracetamol due to the increased risk of high anion gap metabolic acidosis (HAGMA). Patients at particularly high risk are those with severe renal impairment, sepsis or malnutrition, especially if the maximum daily doses of paracetamol are used. After co-administration of flucloxacillin and paracetamol, a close monitoring is recommended in order to detect the appearance of acid-base disorders, namely HAGMA. - The occurrence at the treatment initiation of a feverish generalised erythema associated with pustula may be a symptom of acute generalised exanthematous pustulosis (AGEP). In case of AGEP diagnosis, flucloxacillin should be discontinued and any subsequent administration of flucloxacillin contra-indicated. - The use of flucloxacillin (like other penicillins) in patients with renal impairment does not usually require dosage reduction. In the presence of severe renal failure (creatinine clearance less than 10ml/min), however, a reduction in dose or an extension of dose interval should be considered because of the risk of neurotoxicity and such patients are excluded from the PGD. - Hepatitis and cholestatic jaundice have been reported. These reactions are related neither to the dose nor to the route of administration. - Flucloxacillin should be used with caution in patients with evidence of hepatic dysfunction (excluded from PGD), patients over 50 years or patients with underlying disease all of whom are at increased risk of hepatic reactions. The onset of these hepatic effects may be delayed for up to two months post-treatment. In several cases, the course of the reactions has been protracted and lasted for some months. In very rare cases, a fatal outcome has been reported. - As for other penicillins contact with the skin should be avoided as sensitisation may occur.

	- Patients with a known history of allergy are more likely to develop a hypersensitivity reaction. - Prolonged use of an anti-infective agent may occasionally result in overgrowth of non-susceptible organisms. - Before initiating therapy with flucloxacillin, careful enquiry should be made concerning previous hypersensitivity reactions to β-lactams. Cross-sensitivity between penicillins and cephalosporins is well documented. Serious and occasionally fatal hypersensitivity reactions (anaphylaxis) have been reported in patients receiving β-lactam antibiotics. Although anaphylaxis is more frequent following parenteral therapy, it has occurred in patients on oral therapy. These reactions are more likely to occur in individuals with a history of β-lactam hypersensitivity. If anaphylaxis occurs flucloxacillin should be discontinued and the appropriate therapy instituted. Serious anaphylactic reactions may require immediate emergency treatment with adrenaline (epinephrine). Ensure adequate airway and ventilation and give 100% oxygen. IV crystalloids, hydrocortisone, antihistamine and nebulised bronchodilators may also be required. - Hypokalaemia (potentially life threatening) can occur with the use of flucloxacillin, especially in high doses. Hypokalaemia caused by flucloxacillin can be resistant to potassium supplementation. Regular measurements of potassium levels are recommended during the therapy with higher doses of flucloxacillin. Attention for this risk is warranted also when combining flucloxacillin with hypokalemia-inducing diuretics or when other risk factors for the development of hypokalemia are present (e.g. malnutrition, renal tubule disfunction).
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Where the patient is excluded because of systemic features, suspected necrotising fasciitis or periorbital or facial cellulitis, arrange emergency care (999 or A&E) before the patient leaves the pharmacy. Where excluded for a diabetic foot, lymphoedema, immunosuppression, a bite, suspected DVT or failure of a previous antibiotic, refer to the GP the same day and record that this was done.

Details of the medicine

Name, form and strength of medicine	Flucloxacillin 500mg or 250mg capsules
Legal category	POM
Storage	Store below 25°C in a dry place.
Route/method of administration	Oral administration, taken on an empty stomach (1 hour before or 2 hours after food). Capsules should ideally be taken with a full glass of water (250 ml)
Dose and frequency	500 mg four times a day (every 6 hours), usually for 5 to 7 days depending on clinical response.

Quantity to be administered and/or supplied	28 capsules for 7 days or 20 capsules for 5 days.
Maximum or minimum treatment period	Typically, 5 to 7 days depending on severity and response.
Adverse effects	Nausea, diarrhoea, abdominal discomfort, skin rash. Rare: cholestatic jaundice, anaphylaxis. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● the time the margin of the erythema was marked, the date and time of the booked 48-hour reassessment at this pharmacy, and its outcome ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 2-3 days, if they have severe pain out of proportion to the infection or if the redness and swelling continues to develop beyond the initial presentation, or they become systemically very unwell. Attend the 48-hour reassessment booked at this pharmacy. If the redness spreads beyond the marked edge before then, seek help the same day. Take paracetamol or ibuprofen for pain and fever if not at-risk of HAGMA Drink adequate fluids. Elevate the leg for comfort and to relieve oedema if applicable. Avoid the use of compression garments during acute cellulitis.

	Identify and manage comorbidities (such as diabetes mellitus) that may cause the cellulitis to spread rapidly, or delay healing. Advise on preventative measures to reduce the risk of recurrence, including weight loss (where applicable) and the use of emollients to prevent dry and cracking skin.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) | NICE
- Summary of Product Characteristics for drug

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the

responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD Later versions: see the Version and change record at the end of this document.	28th December 2025

Patient Group Direction

for the supply/administration of

Clarithromycin 250mg or 500mg tablets

for the treatment of

Cellulitis

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Clarithromycin is indicated for the treatment of MILD cellulitis (Eron class I: no signs of systemic toxicity and no uncontrolled comorbidity) of a limb or the trunk in adults aged 18 years and over, caused by susceptible Gram-positive organisms such as Staphylococcus aureus or Streptococcus species. Moderate or severe cellulitis (Eron class II to IV) is not covered by this PGD.
Inclusion criteria	- Adults aged 18 years or older. - MILD cellulitis (Eron class I) of a limb or the trunk: localised erythema, warmth, swelling and pain, with NO fever, NO tachycardia, NO hypotension, NO confusion and NO rapidly spreading margin. - The margin of the erythema has been marked and the time recorded, so that spread can be judged at review. - A reassessment at 48 hours at the supplying pharmacy has been booked before the patient leaves, and the date and time recorded. - No known allergy to macrolides. - Informed consent obtained.
Exclusion criteria	- Known allergy or hypersensitivity to macrolides. - Any sign of systemic illness or sepsis: temperature 38°C or above or below 36°C, heart rate above 90, respiratory rate 20 or above, systolic blood pressure below 100, new confusion, or rigors (call 999 or send to A&E). - Suspected necrotising fasciitis: pain out of proportion to the appearance, rapid spread, skin discolouration, crepitus or blistering (999). - Periorbital, orbital or facial cellulitis (same-day urgent referral). - Cellulitis of a diabetic foot, or in a limb with lymphoedema or chronic venous ulceration (same-day GP referral). - Immunosuppression, including chemotherapy, biologics, long-term oral steroids, or poorly controlled diabetes. - Cellulitis following an animal or human bite, or with fresh water or sea water exposure (different organisms; refer). - Suspected deep vein thrombosis, or redness of both legs (usually not cellulitis; refer). - Abscess requiring drainage or infected wound needing surgical review. - Not improving after 48 hours of an antibiotic, or a second episode at the same site within 3 months (refer). - Pregnant or breastfeeding individuals (use the flucloxacillin arm where there is no penicillin allergy; otherwise refer). - Hepatic dysfunction, or a history of jaundice or hepatic dysfunction associated with clarithromycin. - Clarithromycin is contraindicated in patients with known hypersensitivity to the active substance clarithromycin, to other macrolide antibiotic drugs or to any of the excipients. - Concomitant administration of clarithromycin and ergot alkaloids (e.g.,

	ergotamine or dihydroergotamine) is contraindicated, as this may result in ergot toxicity. - Concomitant administration of clarithromycin and oral midazolam is contraindicated. - Concomitant administration of clarithromycin and lomitapide is contraindicated. - Concomitant administration of clarithromycin and any of the following drugs is contraindicated: astemizole, cisapride, domperidone, pimozide and terfenadine as this may result in QT prolongation and cardiac arrhythmias, including ventricular tachycardia, ventricular fibrillation and torsades de pointes. - Clarithromycin should not be given to patients with history of QT prolongation (congenital or documented acquired QT prolongation) or ventricular cardiac arrhythmia, including torsades de pointes. - Concomitant administration with ticagrelor, ivabradine or ranolazine is contraindicated. - Clarithromycin should not be used concomitantly with HMG-CoA reductase inhibitors (statins) that are extensively metabolised by CYP3A4 (lovastatin or simvastatin), due to the increased risk of myopathy, including rhabdomyolysis. - As with other strong CYP3A4 inhibitors, Clarithromycin should not be used in patients taking colchicine. - Clarithromycin should not be given to patients with electrolyte disturbances (hypokalaemia or hypomagnesaemia, due to the risk of prolongation of the QT interval). - Clarithromycin should not be used in patients who suffer from severe hepatic failure in combination with renal impairment. - Taking any medication with a clinically significant interaction.
Cautions	- Use with caution in patients with renal impairment or history of hepatic issues. - Advise patients to complete the full course and report any side effects. - Consider Clostridioides difficile risk in patients with recent antibiotic use or hospitalisation. - The 48-hour reassessment is performed in person by a pharmacist at the supplying pharmacy and is booked as an appointment before the patient leaves; a phone call is not sufficient. At the reassessment record whether the erythema is within or beyond the mark, the temperature, whether pain has improved, and the decision. Spread beyond the mark, no improvement, deterioration or an unclear diagnosis is a same-day referral. If the patient does not attend, contact them the same day; if they cannot be reached, record the attempt and inform the GP. - Clarithromycin is principally metabolised by the liver. Therefore, caution should be exercised in administering this antibiotic to patients with impaired hepatic function. - Caution should also be exercised when administering clarithromycin to patients with moderate to severe renal impairment

	- Hepatic dysfunction, including increased liver enzymes, and hepatocellular and/or cholestatic hepatitis, with or without jaundice, has been reported with clarithromycin. This hepatic dysfunction may be severe and is usually reversible. Cases of fatal hepatic failure have been reported. Some patients may have had pre-existing hepatic disease or may have been taking other hepatotoxic medicinal products. Patients should be advised to stop treatment and contact their doctor if signs and symptoms of hepatic disease develop, such as anorexia, jaundice, dark urine, pruritus, or tender abdomen. - Pseudomembranous colitis has been reported with nearly all antibacterial agents, including macrolides, and may range in severity from mild to life-threatening. Clostridioides difficile-associated diarrhoea (CDAD) has been reported with use of nearly all antibacterial agents including clarithromycin, and may range in severity from mild diarrhoea to fatal colitis. Treatment with antibacterial agents alters the normal flora of the colon, which may lead to overgrowth of C. difficile. CDAD must be considered in all patients who present with diarrhoea following antibiotic use. Careful medical history is necessary since CDAD has been reported to occur over two months after the administration of antibacterial agents. Therefore, discontinuation of clarithromycin therapy should be considered regardless of the indication. Microbial testing should be performed and adequate treatment initiated. Drugs inhibiting peristalsis should be avoided. - There have been post-marketing reports of colchicine toxicity with concomitant use of clarithromycin and colchicine, especially in the elderly, some of which occurred in patients with renal insufficiency. Deaths have been reported in some such patients. Concomitant administration of clarithromycin and colchicine is contraindicated. - Caution is advised regarding concomitant administration of clarithromycin and triazolobenzodiazepines, such as triazolam, and intravenous or oromucosal midazolam. - Cardiovascular Events: Prolongation of the QT interval, reflecting effects on cardiac repolarisation imparting a risk of developing cardiac arrhythmia and torsades de pointes, have been seen in patients treated with macrolides including clarithromycin. Due to increased risk of QT prolongation and ventricular arrhythmias (including torsades de pointes), the use of clarithromycin is contraindicated in patients taking any of astemizole, cisapride, domperidone, pimozide and terfenadine; in patients who have electrolyte disturbances such as hypomagnesaemia or hypokalaemia; and in patients with a history of QT prolongation or ventricular cardiac arrhythmia. Epidemiological studies investigating the risk of adverse cardiovascular outcomes with macrolides have shown variable results. Some observational studies have identified a rare short-term risk of

	arrhythmia, myocardial infarction and cardiovascular mortality associated with macrolides including clarithromycin. Consideration of these findings should be balanced with treatment benefits when prescribing clarithromycin. - Pneumonia: In view of the emerging resistance of Streptococcus pneumoniae to macrolides, it is important that sensitivity testing be performed when prescribing clarithromycin for community-acquired pneumonia. In hospital-acquired pneumonia, clarithromycin should be used in combination with additional appropriate antibiotics. - Skin and soft tissue infections of mild to moderate severity: These infections are most often caused by Staphylococcus aureus and Streptococcus pyogenes, both of which may be resistant to macrolides. Therefore, it is important that sensitivity testing be performed. In cases where beta–lactam antibiotics cannot be used (e.g. allergy), other antibiotics, such as clindamycin, may be the drug of first choice. Currently, macrolides are only considered to play a role in some skin and soft tissue infections, such as those caused by Corynebacterium minutissimum, acne vulgaris, and erysipelas and in situations where penicillin treatment cannot be used. - In the event of severe acute hypersensitivity reactions, such as anaphylaxis, severe cutaneous adverse reactions (SCAR) (e.g. Acute generalised exanthematous pustulosis (AGEP), Stevens-Johnson Syndrome, toxic epidermal necrolysis and Drug Rash with Eosinophilia and Systemic Symptoms (DRESS), clarithromycin therapy should be discontinued immediately and appropriate treatment should be urgently initiated. - Clarithromycin should be used with caution when administered concurrently with medications that induce the cytochrome CYP3A4 enzyme. - Carefully consider the balance of benefits and risks before prescribing clarithromycin for any patients taking hydroxychloroquine or chloroquine, because of the potential for an increased risk of cardiovascular events and cardiovascular mortality. Furthermore, clarithromycin should be used with caution in the following: • Patients with coronary artery disease, severe cardiac insufficiency, conduction disturbances or clinically relevant bradycardia; • Patients concomitantly taking other medicinal products associated with QT prolongation other than those which are contraindicated - Oral hypoglycaemic agents/Insulin: The concomitant use of clarithromycin and oral hypoglycaemic agents (such as sulfonylurias) and/or insulin can result in significant hypoglycaemia. Careful monitoring of glucose is recommended. - Oral anticoagulants: There is a risk of serious haemorrhage and significant elevations in International Normalized Ratio (INR) and prothrombin time when clarithromycin is co-administered with warfarin.

	INR and prothrombin times should be frequently monitored while patients are receiving clarithromycin and oral anticoagulants concurrently. - Caution should be exercised when clarithromycin is co-administered with direct acting oral anticoagulants such as dabigatran, rivaroxaban, apixaban and edoxaban, particularly to patients at high risk of bleeding. - Long-term use may, as with other antibiotics, result in colonisation with increased numbers of non-susceptible bacteria and fungi. If superinfections occur, appropriate therapy should be instituted. - Attention should also be paid to the possibility of cross resistance between clarithromycin and other macrolide drugs, as well as lincomycin and clindamycin.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Where the patient is excluded because of systemic features, suspected necrotising fasciitis or periorbital or facial cellulitis, arrange emergency care (999 or A&E) before the patient leaves the pharmacy. Where excluded for a diabetic foot, lymphoedema, immunosuppression, a bite, suspected DVT or failure of a previous antibiotic, refer to the GP the same day and record that this was done.

Details of the medicine

Name, form and strength of medicine	Clarithromycin tablets 250mg or 500mg
Legal category	POM
Storage	Store below 25°C in a dry place.
Route/method of administration	By oral administration, twice daily for 5 to 7 days: 500 mg tablets, or 250 mg tablets where the renal dose reduction below applies.
Dose and frequency	500 mg twice daily for 5 to 7 days depending on clinical response. In patients with renal impairment with creatinine clearance less than 30 mL/min, halve the dose to 250 mg twice daily, using 250 mg tablets.
Quantity to be administered and/or supplied	10 tablets for 5 days or 14 tablets for 7 days, of the strength selected in the dose row (500 mg, or 250 mg where the renal dose reduction applies). Record which.
Maximum or minimum treatment period	Typically, 5 to 7 days depending on severity and response.
Adverse effects	The most frequent and common adverse reactions related to clarithromycin therapy are abdominal pain, diarrhoea, nausea, vomiting and taste perversion. Refer to SmPC for full details.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.

Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● the time the margin of the erythema was marked, the date and time of the booked 48-hour reassessment at this pharmacy, and its outcome ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 2-3 days, if they have severe pain out of proportion to the infection or if the redness and swelling continue to develop beyond the initial presentation, or they become systemically very unwell. Attend the 48-hour reassessment booked at this pharmacy. If the redness spreads beyond the marked edge before then, seek help the same day. Take paracetamol or ibuprofen for pain and fever if appropriate. Drink adequate fluids. Elevate the leg for comfort and to relieve oedema if applicable. Avoid the use of compression garments during acute cellulitis. Identify and manage comorbidities (such as diabetes mellitus) that may cause the cellulitis to spread rapidly, or delay healing. Advise on preventative measures to reduce the risk of recurrence, including weight loss (where applicable) and the use of emollients to prevent dry and cracking skin.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources

- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) | NICE
- Summary of Product Characteristics for drug

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Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group	Job title & name of	Signature	Date

signatory	organisation		

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD Later versions: see the Version and change record at the end of this document.	28/12/25

Patient Group Direction

for the supply/administration of

Doxycycline caps 100mg

for the treatment of

Cellulitis

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Doxycycline is indicated for the treatment of MILD cellulitis (Eron class I: no signs of systemic toxicity and no uncontrolled comorbidity) of a limb or the trunk in adults aged 18 years and over, caused by susceptible Gram-positive organisms such as Staphylococcus aureus or Streptococcus species. Moderate or severe cellulitis (Eron class II to IV) is not covered by this PGD.
Inclusion criteria	- Adults aged 18 years or older. - MILD cellulitis (Eron class I) of a limb or the trunk: localised erythema, warmth, swelling and pain, with NO fever, NO tachycardia, NO hypotension, NO confusion and NO rapidly spreading margin. - The margin of the erythema has been marked and the time recorded, so that spread can be judged at review. - A reassessment at 48 hours at the supplying pharmacy has been booked before the patient leaves, and the date and time recorded. - No known allergy to doxycycline or other tetracyclines. - Informed consent obtained.
Exclusion criteria	- Known allergy or hypersensitivity to doxycycline or other tetracycline antibiotics. - Any sign of systemic illness or sepsis: temperature 38°C or above or below 36°C, heart rate above 90, respiratory rate 20 or above, systolic blood pressure below 100, new confusion, or rigors (call 999 or send to A&E). - Suspected necrotising fasciitis: pain out of proportion to the appearance, rapid spread, skin discolouration, crepitus or blistering (999). - Periorbital, orbital or facial cellulitis (same-day urgent referral). - Cellulitis of a diabetic foot, or in a limb with lymphoedema or chronic venous ulceration (same-day GP referral). - Immunosuppression, including chemotherapy, biologics, long-term oral steroids, or poorly controlled diabetes. - Cellulitis following an animal or human bite, or with fresh water or sea water exposure (different organisms; refer). - Suspected deep vein thrombosis, or redness of both legs (usually not cellulitis; refer). - Abscess requiring drainage or infected wound needing surgical review. - Not improving after 48 hours of an antibiotic, or a second episode at the same site within 3 months (refer). - Pregnancy or breastfeeding (doxycycline is contraindicated; use the flucloxacillin arm where there is no penicillin allergy; otherwise refer). - Taking any medication with a clinically significant interaction.
Cautions	- Use with caution in patients with renal impairment or history of hepatic issues (or concurrently receiving potentially hepatotoxic drugs). - Advise patients to complete the full course and report any side effects.

	- Consider Clostridioides difficile risk in patients with recent antibiotic use or hospitalisation. - The 48-hour reassessment is performed in person by a pharmacist at the supplying pharmacy and is booked as an appointment before the patient leaves; a phone call is not sufficient. At the reassessment record whether the erythema is within or beyond the mark, the temperature, whether pain has improved, and the decision. Spread beyond the mark, no improvement, deterioration or an unclear diagnosis is a same-day referral. If the patient does not attend, contact them the same day; if they cannot be reached, record the attempt and inform the GP. - Photosensitivity manifested by an exaggerated sunburn reaction has been observed in some individuals taking tetracyclines, including doxycycline. Patients likely to be exposed to direct sunlight or ultraviolet light should be advised that this reaction can occur with tetracycline drugs and treatment should be discontinued at the first evidence of skin erythema. - Photoonycholysis has also been reported in patients receiving doxycycline. - Benign intracranial hypertension (pseudotumor cerebri) has been associated with the use of tetracyclines including doxycycline. Benign intracranial hypertension (pseudotumor cerebri) is usually transient, however cases of permanent visual loss secondary to benign intracranial hypertension (pseudotumor cerebri) have been reported with tetracyclines including doxycycline. If visual disturbance occurs during treatment, prompt ophthalmologic evaluation is warranted. Since intracranial pressure can remain elevated for weeks after drug cessation patients should be monitored until they stabilize. Concomitant use of isotretinoin or other systemic retinoids and doxycycline should be avoided because isotretinoin is also known to cause benign intracranial hypertension (pseudotumor cerebri). - Microbiological overgrowth The use of antibiotics may occasionally result in the overgrowth of non-susceptible organisms including Candida. If a resistant organism appears, the antibiotic should be discontinued and appropriate therapy instituted. Pseudomembranous colitis has been reported with nearly all antibacterial agents, including doxycycline, and has ranged in severity from mild to life-threatening. It is important to consider this diagnosis in patients who present with diarrhoea subsequent to the administration of antibacterial agents. Clostridium difficile associated diarrhoea (CDAD) has been reported with use of nearly all antibacterial agents, including doxycycline, and may range in severity from mild diarrhoea to fatal colitis. Treatment with antibacterial agents alters the normal flora of the colon leading to overgrowth of C. difficile. C. difficile produces toxins A and B which contribute to the development of CDAD.

	Hypertoxin producing strains of C. difficile cause increased morbidity and mortality, as these infections can be refractory to antimicrobial therapy and may require colectomy. CDAD must be considered in all patients who present with diarrhoea following antibiotic use. Careful medical history is necessary since CDAD has been reported to occur over two months after the administration of antibacterial agents. -Oesophagitis Instances of oesophagitis and oesophageal ulcerations have been reported in patients receiving capsule and tablet forms of drugs in the tetracycline class, including doxycycline. Most of these patients took medications immediately before going to bed or with inadequate amounts of fluid. -Porphyria There have been rare reports of porphyria in patients receiving tetracyclines. -Myasthenia gravis Due to a potential for weak neuromuscular blockade, care should be taken in administering tetracyclines to patients with myasthenia gravis. -Systemic lupus erythematosus Tetracyclines can cause exacerbation of SLE. -Methoxyflurane Caution is advised in administering tetracyclines with methoxyflurane. -Alcohol dependence, alcohol may decrease the half-life of doxycycline.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Where the patient is excluded because of systemic features, suspected necrotising fasciitis or periorbital or facial cellulitis, arrange emergency care (999 or A&E) before the patient leaves the pharmacy. Where excluded for a diabetic foot, lymphoedema, immunosuppression, a bite, suspected DVT or failure of a previous antibiotic, refer to the GP the same day and record that this was done.

Details of the medicine

Name, form and strength of medicine	Doxycycline 100mg capsules
Legal category	POM
Storage	Store below 25°C in a dry place.
Route/method of administration	Oral administration, the capsules should be swallowed with a full glass of water (250ml) in either the sitting or standing position and well before retiring at night to reduce the risk of oesophageal irritation and ulceration. If gastric irritation occurs, it is recommended that Doxycycline capsules be given with food or milk. Studies indicate that the absorption of doxycycline is not notably influenced by simultaneous ingestion of food or milk.
Dose and frequency	200mg on the first day and then 100mg daily. Usual course length of 5 to 7 days depending on clinical response.

Quantity to be administered and/or supplied	6 capsules for 5 days and 8 capsules for 7 days.
Maximum or minimum treatment period	Typically, 5 to 7 days depending on severity and response.
Adverse effects	Refer to SmPC for further detail

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● the time the margin of the erythema was marked, the date and time of the booked 48-hour reassessment at this pharmacy, and its outcome ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 2-3 days, if they have severe pain out of proportion to the infection or if the redness and swelling continues to develop beyond the initial presentation, or they become systemically very unwell. Attend the 48-hour reassessment booked at this pharmacy. If the redness spreads beyond the marked edge before then, seek help the same day. Take paracetamol or ibuprofen for pain and fever if appropriate. Drink adequate fluids. Elevate the leg for comfort and to relieve oedema if applicable. Avoid the use of compression garments during acute cellulitis. Identify and manage comorbidities (such as diabetes mellitus) that may cause the

	cellulitis to spread rapidly, or delay healing. Advise on preventative measures to reduce the risk of recurrence, including weight loss (where applicable) and the use of emollients to prevent dry and cracking skin.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) | NICE
- Summary of Product Characteristics for drug

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Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD Later versions: see the Version and change record at the end of this document.	28/12/25

Version and change record

Version: v007

Issued: 30 September 2026

Supersedes: Version 006, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v006

Issued: 14 September 2026

Supersedes: Version 005, 14 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The version strapline and the statement naming Get Real Health Limited as the owner and authorising organisation now sit under the title on page 1, where every other Get Real Health PGD carries them. In version 005 they followed the guidance summary on page 4. No clinical content changes.

Previous version records

Version: v005

Issued: 14 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its

company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 26: pregnancy and breastfeeding are no longer exclusions in the flucloxacillin arm; flucloxacillin may be supplied where clinically indicated (NICE NG141 first line in pregnancy; SmPC), matching the Skin and Soft Tissue Infection PGD, with a caution to record the indication. The clarithromycin and doxycycline arms keep the exclusion and point to the flucloxacillin arm where there is no penicillin allergy.

Decision 28: a reassessment at 48 hours at the supplying pharmacy, booked before the patient leaves, is now an inclusion criterion in all three arms; the cautions state who performs it, what is recorded and what happens if the patient does not attend; the follow-up advice, the records row and the guidance summary say so.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Flucloxacillin arm: the line 'Breastfeeding individuals unless under appropriate supervision' is removed because it contradicted the exclusion of pregnancy or breastfeeding on the line above; the exclusion stands.

Clarithromycin arm: the truncated exclusion 'Hepatic dysfunction or history' completed.

Clarithromycin arm: route, renal dose reduction and quantity rows now state which tablet strength is used (500 mg, or 250 mg where the renal halving applies) and the phrase 'in more severe infections' is removed.

Change history tables now point to the version and change record for versions after 001.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v002

Issued: 11 September 2026

Supersedes: Version 001, 28 December 2025

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Indication restricted to MILD cellulitis (Eron class I) of a limb or the trunk in all three arms, and defined. The previous version covered 'mild to moderate' cellulitis with 'signs of severe infection' as the only severity exclusion.

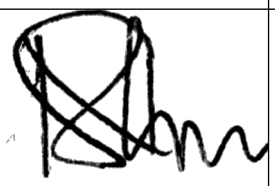
Named exclusions added to all three arms: sepsis and systemic features (999), suspected necrotising fasciitis (999), periorbital or facial cellulitis, diabetic foot, lymphoedema or venous ulceration, immunosuppression, bites and water exposure, suspected DVT or bilateral redness, failure of a previous antibiotic. The 'actions if excluded' row now gives the same-day escalation route.

Marking the margin of the erythema at the consultation is an inclusion requirement, so spread can be judged at review.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Signed on behalf of Get Real Health

Version v007, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.